

Diagnosis and Care Navigator

Educational guidance for evaluation, medication, and support pathways

Scope and Safety

This guide is educational and does not diagnose ADHD. Only licensed clinicians can diagnose and prescribe treatment.

Use this material to prepare for appointments, ask better questions, and navigate options with less confusion.

For immediate crisis support in the U.S., call or text 988.

When to Seek Evaluation

Consider evaluation when symptoms are persistent, cross-setting, and functionally impairing despite self-help attempts.

Bring examples from school, work, and relationships. Concrete impairment examples are more useful than broad labels.

Track overlap factors such as sleep debt, anxiety, depression, and substance use patterns to support differential assessment.

Evaluation Process Breakdown

Most evaluations include clinical interview, symptom history, rating scales, and differential screening for overlapping conditions.

Ask providers how they assess adult ADHD, how they evaluate overlap conditions, and what follow-up looks like after diagnosis.

If concerns are dismissed without assessment reasoning, request clarification and consider second opinions.

Cost-Constrained Care Pathways

Explore community clinics, university psychology clinics, and sliding-scale providers first. Ask about full-cost scope (report + follow-up).

Telehealth can reduce access friction but requires credential clarity and continuity planning.

Avoid services that promise guaranteed diagnosis or hide clinician qualifications.

Medication Education Basics

Medication pathways can include stimulants and non-stimulants. Selection and adjustment are individualized and clinician-guided.

Track dose timing, effect window, appetite, sleep, mood shifts, and rebound effects to improve appointment quality.

Do not self-adjust dosage. Bring concise data summaries and one clear question per visit.

Therapy, Coaching, and Skills Support

CBT can support planning, regulation, and thought-pattern reframing. Coaching supports practical system design and accountability.

Skills groups reduce isolation and reinforce practical behavior change through repeated implementation.

Most people do best with integrated care: medication decisions, behavioral systems, and social support together.

Path-Based Navigation

Path 1: Not diagnosed but struggling - focus on evaluation preparation and self-screening education.

Path 2: Diagnosed and overwhelmed - stabilize daily anchors and reduce system complexity.

Path 3: Trying meds - build tracking discipline and clinical communication scripts.

Path 4: Parent/partner - use shared systems and non-shaming accountability language.

OpenADHD educational resource. Not medical advice.